

Delegating medicines administration

Page last updated: 27 June 2023

Categories: Organisations we regulate

A care worker supporting people with their medicines must be appropriately trained and competent to carry out this task.

Some medicines cannot be routinely administered by a care worker. For example injections (such as insulin) or medicines administered via a feeding tube are clinical or nursing tasks. A registered nurse (RN) can delegate the administration of these medicines to a care worker.

The RN must be confident that the care worker is competent to take on this task. Delegation must always be in the best interest of the person. Providers should also consider how to obtain consent.

Principles and responsibility

The [Nursing and Midwifery Council](#) (NMC) code says registrants must be accountable for their decisions to delegate tasks and duties to other people. It says they must:

- only delegate tasks and duties that are within the other person's competence
- make sure that everyone they delegate tasks to is adequately supervised and supported

- confirm that the outcome of any task they have delegated to someone else meets the required standard

Staff will need extra and more specific training and competency checks before undertaking these tasks. Read more about [regular training and competency expectations](#).

The RN delegating the task must have full confidence that the care worker is competent to carry out the delegated task.

The care worker must:

- understand their limitations
- know when and how to seek help and escalate concerns
- make sure they are comfortable in carrying out the task safely and correctly

This should also include out of hours arrangements.

The care worker should know what to do if the person refuses their medicines. The care worker's responsibilities should be covered:

- as part of their specific training to the delegated task
- in the provider's policy

When a care worker accepts the delegated task, they are responsible for administering the medicines:

- in line with the prescribed instructions and
- as part of a specific and detailed care plan

Periodically the RN should make sure the care worker remains competent to carry out the delegated task.

The RN must make sure that everyone they delegate tasks to is adequately supervised and supported so they can provide safe and compassionate care

Both the RN and care worker should understand accountability, liability and responsibility. They should make a record of their understanding.

The Royal College of Nursing has published some guidance on the principles of delegation. [Find out more](#)

This page is for:

- **adult social care services**
-

Find out more

[Medicines: information for adult social care services](#)

For further advice, contact medicines.enquiries@cqc.org.uk

See also

[Royal College of Nursing: Information on accountability and delegation for all members of the nursing team](#)

[GOV.UK: administration of medicine in care homes](#)

[Skills for Care: Delegated healthcare activities framework](#)

Diabetes mellitus and insulin use in adult social care

Page last updated: 28 September 2023

Categories: Organisations we regulate

Insulin is a hormone that helps to regulate metabolism and lower blood glucose levels.

It is most often prescribed for patients with [type 1 diabetes](#). It is sometimes also used to treat [type 2 diabetes](#) when other methods have been unable to control blood glucose levels.

Many different insulin preparations are available in the UK. There are 3 main groups of insulins:

- fast-acting
- intermediate-acting
- long-acting.

Insulin is usually given by sub-cutaneous injection. There are many devices available for administering insulin. These are often designed to make it easier for patients and carers to administer.

Care planning

You should have a person-centred care plan for anyone using insulin. This should be reviewed regularly and when people's needs change.

This should include:

- an assessment of the support a person may need to manage their diabetes care, including who will administer insulin
- clear information on the person responsible for all aspects of care
- the details of the person(s) responsible for providing any specialist support
- information on the device and the correct equipment needed
- information to support safe administration of insulin, for example rotation of administration site, and safe use and disposal of device and equipment
- information about blood glucose level monitoring, including the frequency and acceptable range and who will carry out this monitoring
- what to do if blood glucose levels are outside of the acceptable range
- the signs and symptoms for low or high blood glucose levels and the appropriate treatment
- when to seek support from the GP, community nurses, diabetes team, or emergency services
- any person-centred dietary requirements
- any person-centred [cultural](#) requirements.

Monitoring blood glucose levels

Some people can do this for themselves. Where they do this, it must be:

- risk assessed
- reviewed at regular intervals
- reviewed if the person's condition changes.

Blood glucose levels will determine whether insulin doses need to change and by how much. Insulin doses may vary for people whose diabetes is difficult to control. Their blood glucose levels will determine how many units they take each time.

There are different types of blood glucose testing kits, which vary in how they are set up and used. Care home staff should have a process to ensure that these are calibrated and fit for purpose in line with the manufacturer's guidance.

Flash glucose monitoring

Some people monitor their blood glucose levels using a flash glucose monitor. This small sensor is worn just under the skin and records glucose levels continuously throughout the day and night.

Whenever necessary, people can scan the sensor to check their blood glucose level.

Staff should be trained and competent to use and support people to use a flash monitor.

Administering insulin

Self-administration

Some people will be able to administer their own insulin.

You need to carry out a risk assessment and record the risk to check that the person can do this safely. Although some people may be able to administer their insulin, they may need support with dialling the correct dose. You must record any support you give with medicines. Ensure the risk assessment includes person-centred medicines support to maximise a person's ability to self-administer. If a person is unable to manage any part of administering their insulin, nurses or trained care staff should administer.

Only trained and competent staff should be responsible for administering insulin or monitoring blood glucose levels. Where a person's diabetes care is stable, a care worker could complete these activities as a [delegated task](#) in accordance with national guidance.

Care staff must receive specialist training to administer insulin as a delegated task. They should be assessed as competent to administer insulin to the named person or people.

Find out more about [delegating medicines administration](#).

Records need to show how much insulin has been given on each occasion. District or community nurses may keep their own records. Care home staff must have a system of ensuring that information regarding insulin administered by district nurses or community nurses is available at the point of transfer of care.

Rotating injection sites

Staff need to rotate and record the injection site to avoid [lipohypertrophy](#). This is also known as lipos - hard lumps that can form if you inject the same place too often.

Common injection sites include upper arms, thighs, buttocks, and abdomen.

Accessories

A variety of hypodermic needles, syringes, lancets, and other accessories can help with administering and monitoring insulin. They are usually available on prescription.

Disposable insulin pens and insulin cartridges for re-usable pens should only be administered using the pen device with the appropriate needle.

Never use a syringe to extract insulin from pen devices or cartridges. This can lead to serious errors. Find out more in this [NHS Patient Safety Alert](#).

If using insulin from a vial, always measure insulin with an appropriate insulin syringe (marked in units/mL).

Disposal

Dispose of sharps in a suitable container and arrange to dispose of these containers appropriately. If visiting healthcare professionals are administering the insulin, ensure there is an agreement regarding the disposal of sharps.

Find out more about [handling sharps in adult social care](#).

Low blood glucose - hypoglycaemia

Insulin can cause low blood glucose levels (hypoglycaemia). The timing of insulin administration and meals is important. It helps to avoid fluctuations in blood glucose levels.

Be aware of the symptoms of low blood glucose levels. They can include:

- hunger
- anxiety or irritability
- palpitations
- sweating
- tingling lips.

If severe, it can lead to convulsions, loss of consciousness, coma and death.

A person-centred care plan should identify:

- the symptoms of a low blood glucose level
- the actions to take if a person's levels are low.

Some people might need to use emergency 'rescue' medicine or food to increase their glucose levels. For example, oral glucose or glucagon injection.

Care staff should have access to information about hypoglycaemia management.

High blood glucose - hyperglycaemia

There are several reasons why high blood glucose levels (hyperglycaemia) may happen. For example, if a person:

- has missed a dose of medicine
- has eaten more carbohydrate than the body or medicines (or both) can process
- is stressed
- is unwell from an infection
- has been over-treated for low blood glucose levels.

High blood glucose levels can cause [diabetic ketoacidosis \(DKA\)](#), which needs urgent treatment.

The person's care plan should specify:

- what blood sugar levels are appropriate for them
- the actions to take if their levels are high.

Managing complications of diabetes

People with diabetes can be more at risk of additional complications such as kidney, foot, and eye problems.

Care home staff should regularly examine the feet of people with diabetes and ensure there are measures to prevent pressure sores.

It is recommended that each person is registered with an optometrist and is offered regular eye tests.

Storing insulin

If not stored correctly, insulin products can lose their effectiveness. As insulin is a protein, it may break down if frozen or left out of the fridge for longer than the manufacturer specifies.

Medicines stored in a fridge should be between 2°C and 8°C.

Find out more about [storing medicines in a fridge](#).

Once the insulin is in use, it can usually be stored at room temperature for a limited period. Record the date when you first open or use an insulin product. This helps when checking how long the insulin has been out of the fridge. For detailed information, refer to the patient information leaflet for each product.

Labelling

Some multi packs of insulin pens are supplied in boxes labelled with the person's name. Sometimes the supplying pharmacist will label the individual pens. If individual pens are not labelled, make sure it is clear who the insulin pens belong to.

Accurate recording

When prescribing, transcribing, or recording insulin, do not abbreviate the word 'unit'. Always write it in full. Abbreviations (such as 'u') can be confused with a zero (particularly if handwritten). This could lead to serious harm.

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Find out more

[Medicines: information for adult social care services](#)

For further advice, contact medicines.enquiries@cqc.org.uk

See also

[Diabetes UK](#)

[Delegating medicines administration](#)

[Storing medicines in fridges in care homes](#)

[NHS Patient safety alert: Risk of severe harm and death due to withdrawing insulin from pen devices](#)

[Handling sharps in adult social care](#)

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My diabetes, my care

Easy read report

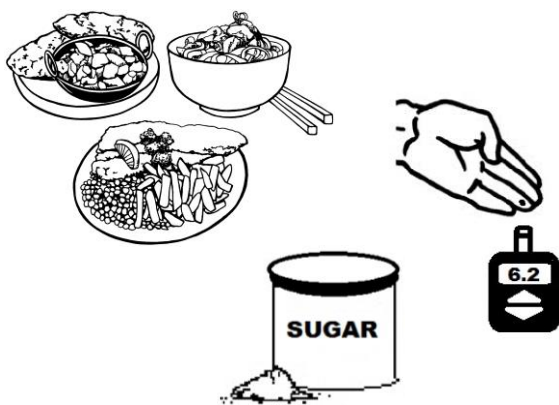
(September 2016)



This is an easy read version of the report called 'My diabetes, my care'. It looks at what happened to people who had care and support in the community to help them look after their diabetes.

It is written by the Care Quality Commission (CQC). We check services like hospitals, doctors' surgeries and care homes to make sure they are giving good health and social care to people.

What is diabetes?

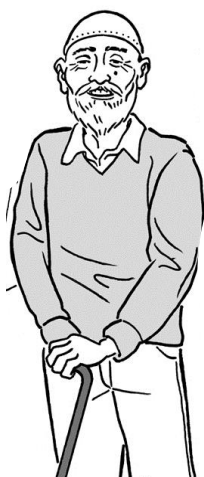


Diabetes is an illness where your blood gets too much sugar from the food you eat.

There are 2 types of diabetes:



People usually get **Type 1 diabetes** before they are 40 years old, and they have to give themselves medicine by injection every day.



People with **Type 2 diabetes** usually get it after they are 40 years old. Most people (9 out of 10 people) who have diabetes have Type 2 diabetes.

What does this report mean for you?

1996



2016



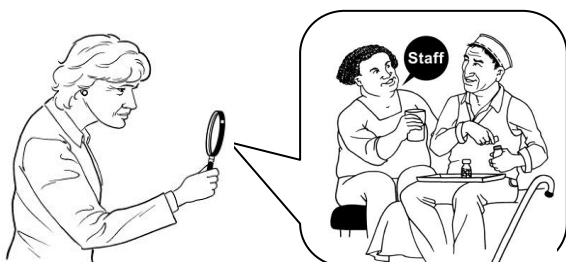
In the last 20 years, the number of adults in England with diabetes has doubled.



Having diabetes affects people's health and how they look after themselves. Some people may get a heart illness, stroke (when the brain doesn't get enough oxygen) or need a part of their body removed in an operation.



It is very important for people with diabetes to receive good care and support to look after their health.



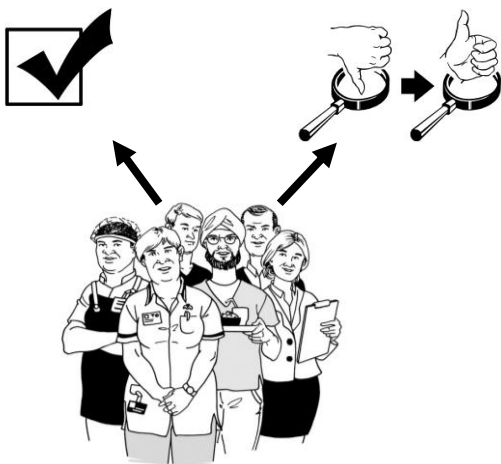
CQC wanted to understand more about what happened to the care of people with diabetes in England. So we looked at how care services work to give good diabetes care.



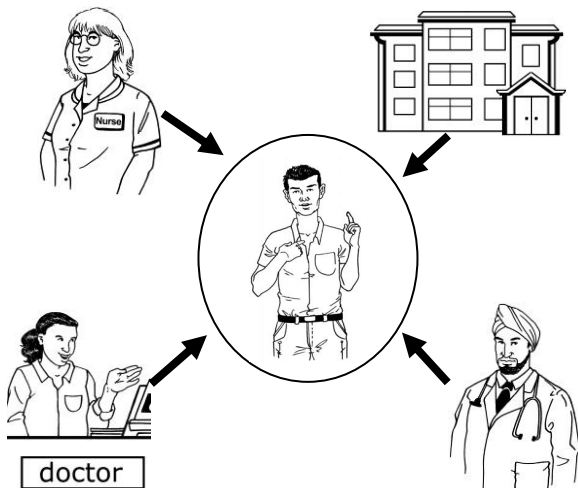
We asked people aged 18 to 65 who have Type 1 or Type 2 diabetes to tell us about what happened when they received care.



We also spoke to commissioners (organisations that look after the care of people), services and staff in 10 different locations about how they give diabetes care in the community.

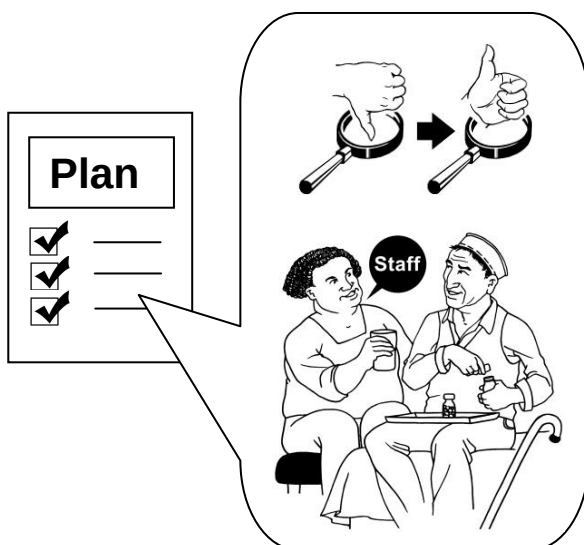


Although many areas are doing a good job, there are still some things that can be improved to make sure everyone has the same good care.



Services must work together to make sure the person with diabetes is at the centre of their care.

What did we find?



1. Most people we spoke to had checks to make sure things will not get difficult for them later on.

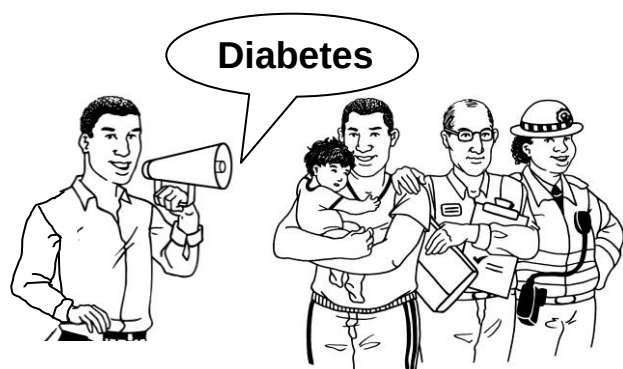
People felt they could talk about their care with staff and have their care explained to them in a way they could understand.

But some people said the care they received could not always be changed to meet their needs.

We said services should have a local plan to improve diabetes care through supporting people to look after their diabetes in a way that best meets their needs.



2. People who were more likely to have Type 2 diabetes were not always found early enough or helped to stop them from getting diabetes.



We said commissioners and services need to make their whole community aware of and understand what diabetes is, and to ask people to have health checks.

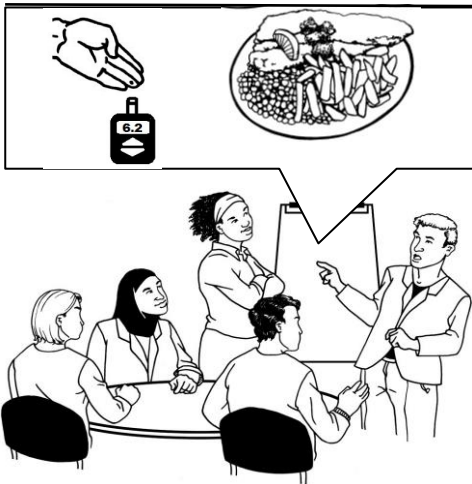


3. Some people need more support to understand and deal with their diabetes, and they need this support whenever they need it.

This support includes those with Type 2 diabetes who may need more help than at first thought.



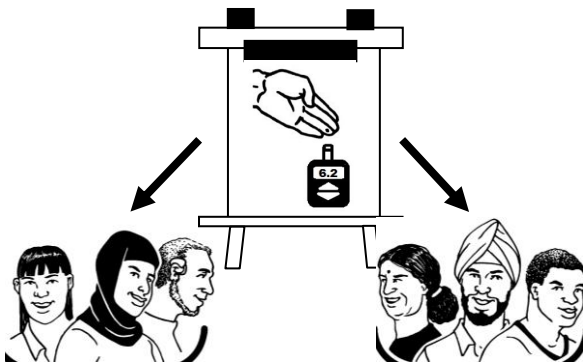
We said services and health staff need to be ready to support and help people understand and deal with their diabetes, and it must be always be included their care.



4. People who attended classes to know more about diabetes felt it improved how they can care for their diabetes better.



But the classes did not meet everyone's needs and there were sometimes no other ways for people to know about diabetes if they did not attend.



We said classes need to cater for everyone, including people from different races and people with a learning disability, so they can gain the knowledge and skills they need to care for their diabetes.



Services should also make better use of technology to support people to manage their diabetes through giving information, helping people want to improve, and letting people check how they are doing.



5. People did not always know or understand the results of their yearly diabetes check-ups.

Very few people had a care plan they could take with them as they moved between services.

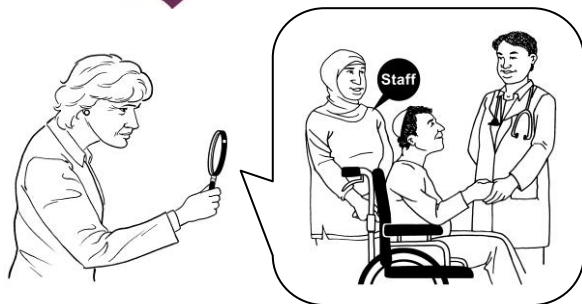
We said that staff should speak to people to understand their needs and help people put a care plan together with their individual needs.

6. Some staff caring for people with diabetes did not know enough about diabetes and some areas had no diabetes training for staff.

This can badly affect people who need other people to help them look after their diabetes.

We said diabetes training should be given for care workers to help them fully support and care for people with diabetes.

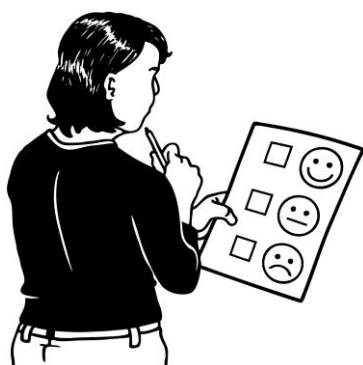
Tell us about your care



When we check GPs and other community services, CQC looks at how illnesses that affect a person for a long time such as diabetes are supported and care for.

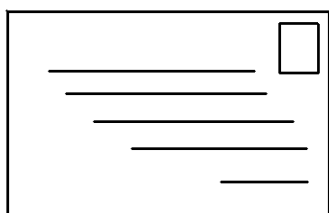


We want to hear about what happened when you receive or have received care.



The information helps us to decide when, where, what to check for and to take action to make sure services improve.

Getting in contact with us



If you would like this report in another format or language, or you would like to tell us something, you can contact us:

Phone: **03000 61 61 61**

Email: enquiries@cqc.org.uk

Website:

www.cqc.org.uk/mydiabetesmycare

Facebook:

CareQualityCommission

Twitter: **@CareQualityComm**

#mydiabetesmycare

Post:

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Handling sharps in adult social care

Page last updated: 16 May 2025

Categories: Organisations we regulate

Sharps injuries are a well-known risk in the health and social care sector.

Sharps contaminated with an infected patient's blood can spread over 20 diseases, including hepatitis B, hepatitis C and HIV (human immunodeficiency virus).

Medical sharps include any device used for a healthcare activity to puncture or lacerate the skin, including:

- needles and anything attached to them, such as syringes
- scalpels and scissors
- lancets.

Broken glass items, such as ampoules or vials, also pose a risk and should be disposed of safely

Legislation

Guidance from the Health and Safety Executive (HSE) says an employer will need to act to manage the risks if workers:

- use sharps to provide care or other services to people
- provide care or other services to people who are likely to use sharps
- are involved in handling such equipment after use, for example in sterile services and waste disposal
- are likely to accidentally come across used sharps, for example during laundering.

Employers are legally required to assess risks from sharps injuries and put appropriate control measures in place.

Health and Safety (Sharp Instruments in Healthcare) Regulations 2013 apply to organisations providing healthcare. This includes:

- nursing homes
- providers delivering healthcare in residential homes or people's own homes.

See [Guidance on the Health and Safety \(Sharp Instruments in Healthcare\) Regulations 2013](#).

Considerations for providers

Avoid the unnecessary use of sharps

Only use sharps when needed.

Needle-free equipment is available for certain procedures. You should use it where it is reasonably practicable to do so.

Use safer sharps

Syringes and needles are available with a shield or cover. This slides or pivots to cover the needle after use to prevent or minimise the risk of accidental injury.

Always work with prescribers and healthcare providers to use safer sharps where possible.

Prevent the recapping of needles

You must not recap needles after use unless the employer's risk assessment has identified that it is required to prevent a risk.

In these limited cases, provide appropriate devices to control the risk of injury to employees. For example, you can use needle-blocks to remove and hold the needle cap and allow safe one-handed recapping.

Safe disposal

Residential homes and care provided in people's own homes

External healthcare professionals may use sharps as part of their routine practice. They should safely dispose of sharps using their own sharps disposal process.

Care workers may use sharps as part of a delegated task. There should be a safe sharps disposal process for staff to follow. This may include disposal through a waste contractor.

Make sure staff are aware how to use sharps. Assess the risk to the person receiving care and to care staff and take appropriate precautions to protect against sharps injury.

Self-administration

People may administer their own medicines and generate sharps waste. For example, people may administer their own insulin or take their own blood glucose readings.

People should dispose of sharps in a dedicated sharps waste bin. This can be prescribed or supplied by the local authority. They should arrange with the local authority to collect these as needed.

When you assess the risk of medicines self-administration include the use, safe storage and disposal of sharps.

Nursing homes

When nurses use sharps in a nursing home, they should dispose of sharps in a dedicated sharps waste bin provided by the clinical waste contractor.

Sharps injury

You should adequately assess the risks from sharps injuries and put appropriate control measures in place.

Train your staff so they know how to work safely and without risk to health with the specific sharps equipment that they or others might use.

Staff should know what action to take if there is a sharps injury or incident.

If staff are injured by sharps at work, they must notify their employer as soon as practicable. The employer must have sufficiently robust arrangements to allow staff to notify them in a timely way. This includes where the employee works out of normal office hours.

If an employee has been injured by a sharp that has or may have exposed them to a blood-borne virus, as the employer, you must:

- make sure the employee has:
 - immediate access to medical advice
 - been offered post-exposure prophylaxis and any other medical treatment, as advised by a doctor
- consider whether the employee should receive counselling.

You must record sharps injuries when notified. You should:

- investigate the circumstances and causes of the incident
 - take any action needed.
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This page is for:

- **adult social care services**
-

Find out more

[Medicines: information for adult social care services](#)

For further advice, contact medicines.enquiries@cqc.org.uk

See also

[Sharps injuries \(Health and Safety Executive\)](#)

[Health and Safety \(Sharp Instruments in Healthcare\) Regulations 2013](#)

[NICE Guidance SC1: Managing medicines in care homes](#)

[NICE Guidance NG67: Managing medicines for adults receiving social care in the community](#)

[Code of Practice on the prevention and control of infections and related guidance](#)

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